AESTHETIC NURSING
POLICY & PROCEDURES, INC.

THE ACCUSATION REVIEW · 2026

What 32 Aesthetic Board Accusations Reveal

The patterns, outcomes and costs behind 32 licensing Board accusations against nurses, NPs, PAs and physicians in aesthetics, and the documents that would have made the difference.

22 of 32
cited missing or insufficient standardized procedures

20 of 32
ended in 3 years' license probation

$7,592
median cost recovery owed to the Board

Cindi Vokey, BSN, RN · Founder & Lead Educator · CA BRN-Approved CE Provider #18009

Education only, not legal advice. Case details are anonymized and simplified.

WHY I DID THIS

Most Board cases aren't decided by what happened in the chair.

In aesthetic nursing since 2007, I have helped providers facing possible licensing issues. Along the way, I kept hearing the same question: What does the Board actually look for?

So I read 32 licensing Board accusations filed against registered nurses, nurse practitioners, physician assistants and physicians working in aesthetics. I charted every citation, every outcome and every dollar of cost recovery. This brief is what I found.

The short version: only 7 of the 32 cases involved documented patient harm. Most began with a complaint, a tip, a customs seizure or an undercover appointment, and they were decided on documentation: standardized procedures, good faith exams and physician oversight.

AT A GLANCE

32
accusations reviewed across RN, NP, PA and physician licenses

22
cited missing or insufficient standardized procedures or P&Ps

7
involved documented patient harm

20
ended in 3 years' probation (revoked, stayed); one more got 35 months

$7,592
median cost recovery owed to the Board (20 cases with an amount)

~3 yrs
typical gap between the conduct and the filed accusation

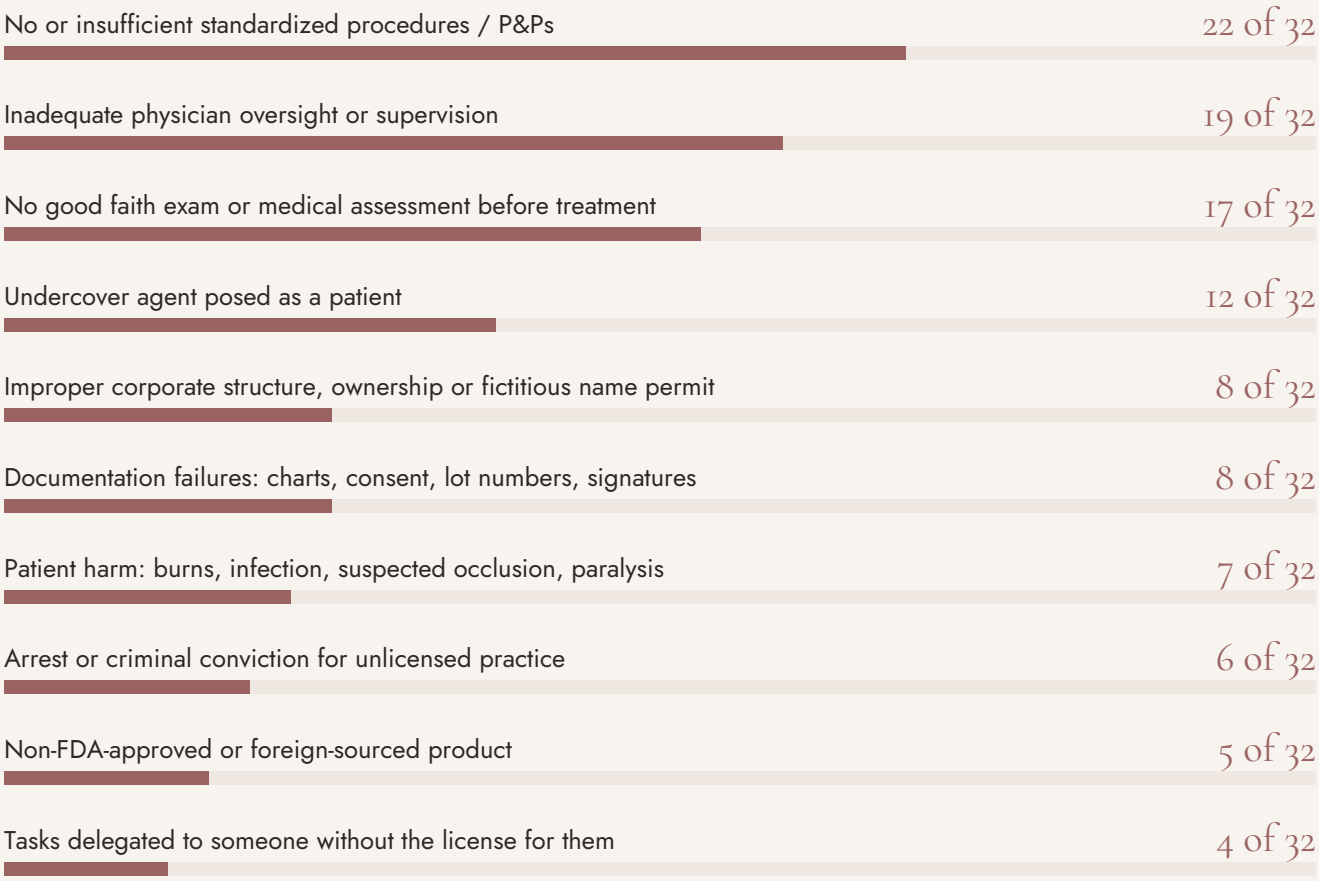
WHO WAS CHARGED

Registered nurses	21
RN / nurse practitioners	2
Nurse practitioners	1
Physicians	6
Physician assistants	2

FINDING 01

The same gaps, again and again

How often each problem appeared across the 32 accusations. A case is counted if the issue appears anywhere in its description or citations, so one case can appear in several rows.



Paperwork decided most cases.	The top three findings are all documentation findings. They exist, or they don't, long before anyone asks.
Generic policies failed.	One nurse submitted a national association's policy set; the Board found it did not meet standardized procedure criteria. Another practice tried to update its P&Ps mid-investigation and still fell short.
Oversight must be real.	An out-of-state medical director covering four medspas. Charts never signed. A medical director with no training in the treatments he oversaw.
The exam must be done by the right person.	Exams by a third party, remote exams with no plan or discussion, or none at all.

FINDING 02

What it costs

An accusation is not the end of the story; it is the start of years of consequences. Here is how the 32 cases resolved.



Each segment = share of 32 cases

3-year probation	20	License revoked, with revocation stayed and 3 years' probation
35-month probation	1	A physician case
Public reproval	5	A permanent, public disciplinary record
Surrendered or revoked	4	Three surrenders, one revocation and suspension
Pending	2	Still open at the time of review

COST RECOVERY OWED TO THE BOARD

$7,592
median across 20 cases with a stated amount

$13,078
average

$74,297
highest single case

$261,567
total across 20 cases

Cost recovery reimburses the Board for its investigation and prosecution. It does not include attorney fees, probation monitoring, required coursework, lost shifts, or the cost of rebuilding a practice's systems.

TIME TO ACCUSATION

Across 29 cases with dates, the typical gap between the conduct and the filed accusation was about three years; 19 took three years or longer, and the longest took five. By then, staff, owners and medical directors had often changed. The records were what remained.

FINDING 03

Six cases, six lessons

Anonymized and simplified. Each case turns on its own facts.

CASE A · DERMAL FILLER · RN

"The Board asked for her policies. There were none to send."

WHAT HAPPENED	A patient complained about dermal filler she said she did not want. The Board requested the nurse's policies and procedures.
WHAT THE BOARD FOUND	None could be provided. The accusation charged incompetence.
OUTCOME	Pending final decision

THE DOCUMENT THAT WOULD HAVE HELPED

Standardized procedures for dermal filler, a written complaint-response protocol, and a signed informed consent specific to the treatment.

CASE B · KYBELLA · RN

"She had policies. They weren't standardized procedures."

WHAT HAPPENED	A patient developed a serious infection after Kybella and reported it to the Board.
WHAT THE BOARD FOUND	Gross negligence and incompetence. The nurse had provided a national association's policy set, which did not meet standardized procedure criteria. The Board also cited missing physician follow-up and an incorrect lot number in the chart.
OUTCOME	Revoked, stayed, 3 years' probation; $14,728 cost recovery

THE DOCUMENT THAT WOULD HAVE HELPED

State-specific standardized procedures approved by the medical director, an adverse-event and physician follow-up protocol, and a lot-tracking log reconciled to the chart.

CASE C · LASER HAIR REMOVAL · RN

"One skipped test spot."

WHAT HAPPENED	A good faith exam was completed. Three months later, laser hair removal at an incorrect setting burned the patient.
WHAT THE BOARD FOUND	Incompetence and unprofessional conduct for failing to perform a test spot.
OUTCOME	Revoked, stayed, 3 years' probation; $10,000 cost recovery

THE DOCUMENT THAT WOULD HAVE HELPED

A laser standardized procedure that requires and documents a test spot, a device-settings log, and a signed provider competency for the device.

CASE D · LASER · OUT-OF-STATE MEDICAL DIRECTOR

"A name on a contract is not oversight."

WHAT HAPPENED	The medical director lived in another state and oversaw four medspas. Nurses performed laser treatments without physician orders, supervision or exams.
WHAT THE BOARD FOUND	Gross negligence, practicing medicine without a license, and aiding and abetting, because the nurse managed staff while knowing supervision was not happening.
OUTCOME	Public reproval

THE DOCUMENT THAT WOULD HAVE HELPED

A medical director agreement with defined supervision duties, a chart-review log, and good faith exam records for every patient.

CASE E · NEUROTOXIN AND FILLER · RN

"It started with a package at customs."

WHAT HAPPENED	The FDA intercepted fillers and neurotoxin shipped from overseas to a nurse's home. An undercover agent then booked an appointment; the exam was done remotely.
WHAT THE BOARD FOUND	Unprofessional conduct and incompetence for obtaining non-FDA-approved products, advertising medical treatments, and practicing without standardized procedures.
OUTCOME	Revoked, stayed, 3 years' probation; $18,688 cost recovery

THE DOCUMENT THAT WOULD HAVE HELPED

A product sourcing and purchasing policy, a good faith exam policy, and an advertising review policy.

CASE F · TWO NURSES, ONE PRACTICE · RN

"The update came too late."

WHAT HAPPENED	After a complaint about oversight, the Board sent an undercover agent. An RN assessed and confirmed the treatment plan; no physician was involved. Agents seized product.
WHAT THE BOARD FOUND	The practice was not a medical corporation and had no fictitious name permit. P&Ps were not in place, and an attempt to update them failed to meet criteria. Both nurses were charged.
OUTCOME	Both: revoked, stayed, 3 years' probation; $5,571 and $6,726 cost recovery

THE DOCUMENT THAT WOULD HAVE HELPED

A corporate structure review before opening, and standardized procedures that meet state criteria from day one.

SELF-AUDIT

15 questions to ask this week

Every item below maps to a finding in the 32 accusations. Any item you can't check with confidence is worth a closer look.

- ☐ 01 Written standardized procedures exist for every treatment we offer, meet our state's criteria, and are signed by the medical director.
- ☐ 02 Our P&Ps were written for our state and scope, not adapted from a generic or association template.
- ☐ 03 Every patient has a documented good faith exam by a physician, NP or PA before treatment begins.
- ☐ 04 Remote or telehealth exams include a documented plan and communication with the treating nurse.
- ☐ 05 Our medical director agreement defines supervision duties, and chart reviews are logged.
- ☐ 06 Our medical director is trained in, and available for, the treatments being supervised.
- ☐ 07 Every chart includes medical history, treatment-specific consent, product, dose, and correct lot number.
- ☐ 08 Laser and energy device protocols require and document test spots and settings.
- ☐ 09 Every provider has signed competencies for each treatment they perform.
- ☐ 10 Adverse events trigger a written protocol, including physician follow-up and documentation.
- ☐ 11 All products are purchased through a legitimate U.S. supply chain, under our own license.
- ☐ 12 Our corporate structure, ownership and fictitious name permit meet our state's rules.
- ☐ 13 No task is delegated to anyone without the license to perform it (estheticians, LVNs, assistants).
- ☐ 14 Our website and social media claims have been reviewed for advertising compliance.
- ☐ 15 If the Board requested our P&Ps today, we could send them within 24 hours.

Prefer to answer online? Take the 2-minute readiness self-check at aestheticnursingpnp.com

NEXT STEPS

Fix it before anyone knocks.

Here is how I can help, wherever you stand.

Compliance Readiness Audit	$950 one-time	Pre-audit document review, a 90-minute walkthrough with the owner and medical director, and a scored gap report. The fee is credited toward a Custom Manual booked within 60 days.
Custom State-Specific Manual	See pricing in Manual Builder	Standardized procedures, consent forms, charting templates, patient instructions and provider competencies for each treatment, with two team training sessions.
Licensing & Probation Support	By consult	For nurses and providers facing a Board complaint, investigation or accusation, or on probation. Coordinated with your attorney.
Continuing Education	From $40	From Complaint to Accusation (2 CE), Good Faith Exam (1 CE), Standardized Procedures (1 CE), and Guidelines for a Compliant Practice: Pillars of Success (3 CE). CA BRN-Approved CE Provider #18009.

BOOK A CONSULT

Cindi Vokey, BSN, RN

aestheticnursingpnp.com

info@aestheticnursingpnp.com · 508-498-6604

Courses: aestheticnursingces.com

About this review. Summaries are drawn from 32 publicly filed licensing Board accusations reviewed by Cindi Vokey, BSN, RN, and are anonymized and simplified. Counts reflect whether an issue appears anywhere in a case's description or citations; a case can appear in several categories. Cost figures reflect the cost recovery stated in each accusation. Each case turns on its own facts. This brief is education, not legal advice; consult a healthcare attorney about your situation.